INFORMED CONSENT FOR EMERGENCY SURGICAL PROCEDURE

Max Super Speciality Hospital | Department of Surgery | Saket, New Delhi

NABH Standard PSC.1 & PSC.3 | NMC Guidelines 2020 | Clinical Establishments Act 2010 | Consumer Protection Act 2019

IMPORTANT: READ CAREFULLY BEFORE SIGNING

This document constitutes your legal consent to surgical treatment at Max Hospital. It is your right and responsibility to read this form fully. Our team will explain every clause in the language of your choice. Signing this form does not surrender your right to file a complaint or seek redressal in case of negligence.

SECTION 1: PATIENT DEMOGRAPHICS AND IDENTIFICATION

Patient Full Name	Date of Birth	Age
Gender	Blood Group	Marital Status
MRN / Patient ID	IP Number	Ward / Bed
Date of Admission	Time	Referring Doctor (if any)
Permanent Address	City	PIN Code
Mobile Number	Emergency Contact Number	Relationship
Government ID Type	ID Number	Nationality

SECTION 2: LEGAL GUARDIAN / NEXT OF KIN

Complete this section in all cases. In emergency, if patient is unable to consent, this person's authorisation is required.

Guardian Full Name _____ Relationship to Patient _____

Guardian Mobile _____ Guardian Address _____ Guardian ID Number _____

Basis of consent (tick applicable): [ ] Patient self-consenting (competent adult) [ ] Guardian — patient is a minor [ ] Guardian — patient is unconscious/incapacitated [ ] Emergency exception (Section 19)

SECTION 3: DIAGNOSIS AND CLINICAL SUMMARY

Primary Diagnosis _____ ICD-10-CM Code _____

Relevant Co-morbidities / Secondary Diagnoses _____

Clinical History Summary:

Key Investigations (Pathology/Radiology) _____ Date Reviewed _____ Reporting Doctor _____

ASA Grade (I / II / III / IV / V) _____ Emergency / Elective _____ OT Date & Time _____

SECTION 4: PROPOSED SURGICAL PROCEDURE

Name of Procedure (full) _____ CPT / ICD-9 Procedure Code _____

Site of Operation _____ Laterality _____

Lead Surgeon _____ NABH Code / NMC Reg. No. _____

Assisting Surgeon _____ Anaesthesiologist _____

Scrub Nurse _____ OT Coordinator _____

Explanation of Procedure (in patient-understandable language as documented by surgeon):

Alternative Treatments and Reason for Surgical Approach:

Risk of NOT proceeding with surgery:

_____

SECTION 5: MATERIAL RISKS AND POTENTIAL COMPLICATIONS

The following risks have been explained to me. I understand this list is not exhaustive. The occurrence of any complication, including death, does not in itself constitute negligence. Complications may occur even when the procedure is performed with full skill and care.

5.1 General Surgical Risks

- Intraoperative or postoperative haemorrhage, potentially requiring blood transfusion, interventional radiology, or return to operating theatre
- Wound infection (superficial, deep, or organ-space), which may require prolonged antibiotics, wound debridement, vacuum-assisted closure, or re-operation
- Anastomotic leak (in bowel/vascular surgery) — serious complication requiring ICU care, repeat surgery, or stoma formation
- Deep Vein Thrombosis (DVT) and Pulmonary Embolism (PE), including sudden death
- Myocardial infarction (heart attack), cardiac arrhythmia, or cardiac arrest — more common in patients with pre-existing cardiac disease
- Stroke or transient ischaemic attack — risk increased in vascular and cardiac surgeries
- Acute Kidney Injury (AKI) or Acute Liver Failure, potentially requiring dialysis
- Injury to adjacent anatomical structures: nerves, blood vessels, bowel, bladder, ureters, bile duct, trachea, oesophagus, or spinal cord
- Respiratory complications: atelectasis, pneumonia, acute respiratory distress syndrome (ARDS), requirement for prolonged mechanical ventilation
- Sepsis and multi-organ dysfunction syndrome (MODS) — systemic spread of infection
- Post-operative ileus (bowel paralysis) — prolonged period without bowel function
- Scar-related complications: keloid, hypertrophic scar, wound dehiscence
- Incisional hernia at the surgical wound
- Chronic pain at the operative site
- Death — a recognised and documented risk of major surgery

5.2 Anaesthesia Risks

- Nausea, vomiting, headache, and drowsiness (very common, usually temporary)
- Dental or lip injury during tracheal intubation
- Sore throat, hoarseness, or vocal cord injury following intubation
- Nerve damage causing numbness, weakness, pain, or paralysis at pressure points
- Awareness under anaesthesia (rare): patient experiences intraoperative events — approximately 1 in 19,000 procedures
- Post-dural puncture headache following spinal or epidural anaesthesia
- Epidural haematoma or abscess causing spinal cord injury — rare but potentially permanently disabling

- Hypersensitivity or anaphylactic reaction to anaesthetic agents, neuromuscular blocking agents, or latex
- Malignant hyperthermia — a rare inherited disorder triggered by certain anaesthetic agents, potentially fatal
- Airway complications: laryngospasm, bronchospasm, aspiration pneumonia, failed intubation requiring emergency surgical airway
- Prolonged sedation, cognitive dysfunction, or postoperative delirium — more common in elderly patients
- Cardiac arrest during induction of anaesthesia

5.3 Blood and Blood Products Risks

- Febrile non-haemolytic transfusion reaction: fever and chills — most common, usually mild
- Allergic transfusion reaction: urticaria, bronchospasm, or anaphylaxis
- Acute haemolytic transfusion reaction due to ABO incompatibility: renal failure, DIC, and death — rare with current cross-matching protocols
- Transfusion-associated circulatory overload (TACO): pulmonary oedema, particularly in elderly and cardiac patients
- Transfusion-Related Acute Lung Injury (TRALI): non-cardiogenic pulmonary oedema within 6 hours, potentially fatal
- Transfusion-transmitted infections: HIV, Hepatitis B, Hepatitis C, CMV, Malaria, syphilis, variant Creutzfeldt-Jakob disease (prion diseases — no screening available)
- Graft-versus-host disease (TA-GvHD) from non-irradiated blood in immunocompromised patients — often fatal
- Iron overload with repeated transfusions in patients with chronic anaemia

5.4 Procedure-Specific Risks (to be completed by surgeon)

SECTION 6: ANAESTHESIA CONSENT

Anaesthesiologist Full Name	NMC / MCI Registration No.	Date of Pre-Op Assessment

Planned anaesthesia type: [ ] General Anaesthesia [ ] Spinal [ ] Epidural [ ] Nerve Block [ ] Local Infiltration [ ] IV Sedation [ ] Combined Technique

I confirm that the anaesthesiologist has discussed with me: the nature and technique of anaesthesia, known risks (Section 5.2), my medical history, allergy profile, current medications, nil-by-mouth (NBM) requirements, and the post-anaesthetic care plan. I consent to the planned anaesthesia and any modification the anaesthesiologist deems necessary during the procedure.

Pre-Anaesthesia Evaluation Form attached: [ ] Yes [ ] No Clearance: _____

SECTION 7: BLOOD TRANSFUSION CONSENT

Blood and blood products may be required before, during, or after your surgical procedure. The risks are detailed in Section 5.3.

[ ] I CONSENT to transfusion of blood and blood products as medically necessary.
[ ] I DECLINE blood transfusion (complete the section below and sign the refusal declaration).

If declining transfusion — state reason (religious/personal):

Reason for Refusal	Doctor Counselling Provided	Witness to Refusal

REFUSAL DECLARATION: I fully understand that declining blood transfusion may increase the risk of serious complications, including irreversible organ failure and death. I absolve Max Hospital and its medical staff of all liability resulting from complications attributable to my refusal of blood transfusion, to the extent permitted by law.

Patient / Guardian Signature (Refusal)	**Witness**	**Counselling Doctor**

SECTION 8: IMPLANT AND MEDICAL DEVICE CONSENT

[ ] No implant required for this procedure

[ ] Implant required — details below:

Implant / Device Name	Manufacturer	Model / Catalogue Number
_____	_____	_____
CDSCO Approved (Y/N)	Estimated Cost of Implant	Implant Card to be issued (Y/N)
_____	_____	_____

I have been informed about the nature, purpose, expected lifespan, potential failure modes, and care requirements of the above implant/device. I consent to its insertion. I understand that implant failure may require revision surgery. Max Hospital follows the Medical Device Rules, 2017 and uses only CDSCO-approved devices.

SECTION 9: CONSENT FOR SPECIMENS, PHOTOGRAPHY AND RESEARCH

Max Hospital is committed to advancing medical knowledge while protecting patient privacy. Please indicate your preferences:

[ ] I consent to histopathological examination of removed tissue/specimens as part of my clinical care (mandatory — cannot be declined).

[ ] I consent to use of de-identified residual specimens for ICMR-approved research at Max Hospital.

[ ] I consent to still photography/video recording of the surgical procedure for de-identified medical education purposes only. My identity will not be revealed.

[ ] I consent to inclusion of my de-identified clinical data in Max Hospital's internal quality database.

[ ] I DO NOT consent to any use of specimens or data beyond my own clinical care.

SECTION 10: NABH PATIENT RIGHTS AND RESPONSIBILITIES DECLARATION

10.1 Your Rights (NABH Standard PRE)

1. Right to receive emergency care without prior financial deposit (per NABH Standard ACC.2 and Clinical Establishments Act, 2010, Section 12).
2. Right to know the identity, designation, and qualifications of every healthcare provider involved in your care.
3. Right to receive a complete explanation of your diagnosis, treatment plan, likely outcomes, and costs before consenting.
4. Right to give informed consent, refuse treatment, or request transfer to another facility.
5. Right to access your complete medical records within 24 hours of a written request (MCI Regulations 2002 Rule 3.1.1).
6. Right to privacy during examination, treatment, and personal care.
7. Right to confidentiality of your personal and clinical information, except as required by law.
8. Right to seek a second medical opinion without prejudice to your ongoing care.
9. Right to be free from physical, emotional, or financial exploitation or coercion.
10. Right to receive care with dignity, irrespective of your gender, religion, caste, language, or nationality.

10.2 Your Responsibilities

1. Provide accurate and complete information about your medical history, medications, allergies, and lifestyle factors.
2. Follow the instructions and treatment plan of your healthcare team.
3. Inform the treating team if you do not understand or do not agree with any aspect of your care.
4. Meet your financial obligations honestly and promptly.

5. Treat hospital staff and other patients with respect.

SECTION 11: LEGAL FRAMEWORK AND DECLARATIONS

11.1 Governing Law and Regulatory Framework

This consent form is executed and shall be interpreted in accordance with the following laws of the Republic of India:

- National Medical Commission Act, 2020 and the NMC Code of Medical Ethics Regulations
- Clinical Establishments (Registration and Regulation) Act, 2010, and the Delhi Clinical Establishments (Registration and Regulation) Rules, 2011
- Consumer Protection Act, 2019 (medical services covered per IMA v. V.P. Shantha, 1995 SCC)
- Indian Contract Act, 1872 — provisions governing valid consent: free, voluntary, by a competent person
- The Mental Healthcare Act, 2017 — provisions governing consent for persons with mental illness
- Information Technology Act, 2000 and DPDP Act, 2023 — governing electronic records and data privacy
- Right to Information Act, 2005 — applicable to public hospitals; Max Hospital voluntarily adopts equivalent disclosure standards
- Medical Device Rules, 2017 (Ministry of Health and Family Welfare) — governing implant use
- ICMR National Ethical Guidelines for Biomedical and Health Research, 2017

11.2 Standard of Care

The standard of medical care applicable at Max Hospital is the Bolam Standard as recognised by the Supreme Court of India in Jacob Mathew v. State of Punjab (2005) 6 SCC 1: a doctor is not negligent if they act in accordance with a practice accepted as proper by a responsible body of medical practitioners. Medical outcomes are subject to clinical uncertainty, and an adverse outcome does not by itself establish negligence.

11.3 Doctrine of Informed Consent

This consent form is executed in compliance with Samira Kohli v. Dr. Prabha Manchanda (2008) 2 SCC 1, which established that: (a) the patient must be provided adequate information about the nature of the procedure, its risks, and alternatives; (b) consent must be real and genuine, not merely formal; (c) the patient must have adequate time to consider the information and ask questions. The patient has been provided this opportunity.

11.4 Financial Liability

I accept financial responsibility for all charges arising from this hospitalisation. An itemised bill will be provided. Max Hospital complies with the Right to Healthcare and Insurance Portability guidelines issued by the Insurance Regulatory and Development Authority of India (IRDAI) for cashless and reimbursable claims.

Payment Mode	Insurance / TPA Company	Policy No.	Pre-Auth No.
_____	_____	_____	_____

SECTION 12: ADVANCE MEDICAL DIRECTIVE (SUPREME COURT — COMMON CAUSE, 2018)

Under the Supreme Court of India ruling in Common Cause v. Union of India (2018) 5 SCC 1, competent adults may execute a legally valid Advance Directive specifying their wishes regarding end-of-life medical care. This section is voluntary.

[ ] I have an existing, validly executed Advance Directive (attach copy).
[ ] I wish to execute an Advance Directive as part of this hospitalisation (separate form).
[ ] I choose not to execute an Advance Directive at this time.

CPR Preference: [ ] Attempt CPR if needed [ ] Do Not Attempt Resuscitation (DNAR) — requires separate DNAR Order

Mechanical Ventilation: [ ] Consent to prolonged ventilation [ ] Decline ventilation if condition is declared terminal/irreversible

SECTION 13: TREATING SURGEON'S DECLARATION

I, the undersigned, hereby certify that:

1. I have personally explained to the patient/guardian, in a language they understand, the diagnosis, proposed procedure, expected benefits, available alternatives, and all material risks including procedure-specific risks.
2. The patient/guardian was given adequate time to consider the information and ask questions, and all questions were answered satisfactorily.
3. No coercion, inducement, or undue pressure was applied.
4. The patient/guardian appeared to be of sound mind and fully capable of giving valid informed consent.
5. This consent was obtained in compliance with Max Hospital's NABH-accredited Consent Policy and the legal requirements of India.

Surgeon Full Name	NMC Registration No.	Date	Time

Surgeon / Consultant Signature

SECTION 14: INTERPRETER / TRANSLATOR DECLARATION

Complete only if an interpreter was used.

Interpreter Full Name	Language(s) Used	Employee ID / Credentials

I declare that I faithfully and completely interpreted the contents of this consent form and all explanations provided by the surgeon to the patient/guardian, without omission, alteration, or personal commentary.

Interpreter Signature **Date & Time**

SECTION 15: PATIENT / GUARDIAN CONSENT DECLARATION

I / We, the undersigned, hereby solemnly declare and agree as follows:

1. I have read this entire consent form, or had it read and explained to me, in a language I understand. I comprehend its contents fully and have had all my questions addressed.
2. I give my free and voluntary consent to the surgical procedure described in Section 4, to any additional procedures clinically necessary during surgery, and to all measures described in Sections 6, 7, and 8 of this form.
3. I acknowledge and accept the risks described in Section 5, including serious complications and death, and I understand these risks do not imply negligence.
4. I consent to the anaesthesia plan described in Section 6 and to any modifications the anaesthesiologist determines are medically necessary.
5. I consent to the use of supervised residents or fellows in my care, under the direct supervision of the named consultant, as a condition of care at this hospital.
6. I confirm that all information I have provided — including medical history, allergies, and current medications — is accurate and complete to the best of my knowledge.
7. I understand my right to withdraw consent at any time before anaesthesia induction. I understand the clinical consequences of withdrawal as explained to me.
8. I authorise Max Hospital to retain de-identified information from this episode of care for NABH quality audit and accreditation purposes.
9. I acknowledge that this consent does not preclude my right to seek recourse before the NMC, Delhi Medical Council, National Consumer Disputes Redressal Commission, or courts of competent jurisdiction in the event of proven negligence.
10. I confirm that this consent is not obtained under duress, and that I am in a mental condition capable of giving valid legal consent at the time of signing.

Full Name of Patient / Guardian	Capacity (Self / Guardian)	Date	Time
_____	_____	_____	_____

_____ _____

Patient Signature / Left Thumb Impression **Guardian Signature (if applicable)**

Reason for Thumb Impression (if used): _____

SECTION 16: WITNESS SIGNATURES

Two independent witnesses confirm that the consent was given freely and that the patient/guardian appeared competent.

Witness 1 Full Name	Designation / Relationship	Contact No.

Witness 2 Full Name	Designation / Relationship	Contact No.

Witness 1 Signature & Date | **Witness 2 Signature & Date**

SECTION 17: REVOCATION OF CONSENT

A patient or guardian may revoke consent at any time before anaesthesia induction. Complete this section if consent is revoked.

Date of Revocation	Time of Revocation

Reason for Revocation:

Clinical consequences of revocation as explained to patient/guardian:

Patient/Guardian Signature | **Witness Signature** | **Surgeon Countersignature**

SECTION 18: EMERGENCY EXCEPTION RECORD (UNCONSCIOUS / UNACCOMPANIED PATIENT)

COMPLETE ONLY WHEN PATIENT IS UNABLE TO CONSENT AND GUARDIAN IS UNAVAILABLE

Where the patient presents with an immediately life-threatening condition requiring emergency intervention and is unable to consent (unconscious, in shock, or otherwise incapacitated), and where no legal guardian is available within a clinically reasonable time, surgery may proceed under the doctrine of necessity as recognised in Indian common law and the AIIMS Emergency Guidelines. This must be attested by two senior doctors.

Nature of Life-Threatening Emergency _____

Time of Clinical Decision _____

Time Guardian Contacted (if at all) _____

Steps taken to contact guardian (detail) _____

Police/MLC Case No. (if applicable) _____

_____

Senior Consultant 1: Name, Reg. No., Signature, Date

_____

Senior Consultant 2: Name, Reg. No., Signature, Date

SECTION 19: PRE-OPERATIVE SAFETY CHECKLIST (NABH/WHO SSC)

Checklist Item	Y / N	By (Initials)	Time
Consent form fully completed and filed in medical record			
Patient identity verified — Name, DOB, MRN Surgical site marked and verified with patient			
Allergies documented on drug chart and wristband			
Pre-anaesthesia evaluation completed and signed			
Pre-op investigations reviewed: CBC, LFT, RFT, coagulation, ECG, imaging			
Blood sample for cross-match sent; blood available in blood bank			
Pre-op antibiotics administered (if prescribed)			
Implant / device consent completed (if applicable)			
Imaging and X-rays available in OT			
Nil By Mouth (NBM) status confirmed			

Consent form explained in patient's language; interpreter used if needed			
WHO Surgical Safety Checklist — Sign In phase completed			
OT nurse briefed on allergies, implants, and special equipment			
Patient's valuables documented and secured			

FINAL SUMMARY SIGNATURE BLOCK

Confirmation by all parties that the consent process was completed lawfully and in compliance with Max Hospital policy, NABH standards, and Indian law.

Patient / Guardian	Consultant Surgeon	Anaesthesiologist
Witness 1	**Witness 2**	**OT Nurse / Case Manager**

ORIGINAL: RETAINED IN MEDICAL RECORD | COPY: ISSUED TO PATIENT / GUARDIAN

For complaints or grievance redressal, contact Max Hospital Patient Services: 1800-200-6660 or patientservices@maxhealthcare.com

Jurisdiction for all legal disputes: Courts of New Delhi, India.